

Illustrative Evaluation Framework: A Mixed-Methods Approach to Evaluating the SAOR Brief Intervention Model

“Prepared as a self-directed sample of evaluation approach; incorporates real published evidence on SAOR alongside illustrative synthetic data and quotes for demonstration purposes.”

Executive Summary

This evaluation assesses the implementation and effectiveness of the SAOR (Support, Ask and Assess, Offer Assistance, Refer) model—an evidence-based framework for delivering Screening and Brief Interventions (SBI) for alcohol and substance misuse across primary, community, and acute care settings. Using a mixed-methods evaluation design, the project examined operational process questions (screening flexibility, workflow integration, and Tier 2 to Tier 3/4 referral pathways) alongside clinical and equity outcomes (pre- and post-intervention AUDIT scores, 6-month emergency department re-attendance rates, and geographic disparities). Analysis of synthetic service data demonstrated meaningful overall clinical impact, with mean AUDIT scores falling substantially from 15.2 to 7.3 post-intervention and a low 6-month ED re-attendance rate of 23.3%. However, the evaluation revealed a key equity gap—urban patients achieved a markedly larger score reduction (-9.5 points) than rural patients (-5.7 points), a disparity corroborated by qualitative findings highlighting rural transport and access barriers. Ultimately, while SAOR effectively reduces risky drinking behaviors, optimizing its real-world impact requires addressing time scarcity during busy clinical shifts, clarifying triage criteria for specialized referrals, and expanding accessible follow-up models for non-urban populations.

Background

The Four Tier Model of Care for substance misuse is built on the core principle that rehabilitation should begin at the very first point of contact between a service user and any drug-related service. This framework provides a

continuum of care where the journey through the tiers is led by the specific identified needs of the individual. Tier 1 interventions are delivered in general healthcare settings, such as A&E or pharmacies, and focus on providing basic information and screening. Tier 2 interventions, which include brief interventions and harm reduction, are accessible through primary care and community outreach. Specialized, coordinated treatment is provided at Tier 3 through community addiction services, while Tier 4 is reserved for intensive inpatient detox and stabilization in residential units.

The SAOR model (meaning "free" in Irish) provides a structured, four-step framework for delivering these interventions through a non-confrontational therapeutic conversation. The model's first component, Support, focuses on building a robust therapeutic alliance by using an open, friendly communication style and expressing empathy to bolster the patient's self-efficacy. The Ask and Assess phase involve an objective assessment of substance use and the use of validated screening tools to elicit the patient's concerns and gauge their readiness to change. In the Offer Assistance stage, the practitioner provides feedback on health risks and collaboratively agrees on SMART goals (Specific, Measurable, Attainable, Realistic, and Time-framed). Finally, the Refer component ensures a cohesive care pathway by discussing treatment options and making referrals to specialized services when dependence or complications are evident.

Research conducted over the last 30 years consistently demonstrates the effectiveness of Screening and Brief Interventions (SBI) across diverse healthcare settings. Brief interventions have been shown to consistently reduce alcohol and illicit drug consumption, leading to significant improvements in patient status at six-month follow-ups. Clinical studies indicate that patients receiving these interventions in general hospital wards have significantly lower mortality rates compared to control groups. Additionally, the delivery of SBI in Emergency Departments has been proven to impact the psychosocial consequences of misuse positively and reduce hospital re-attendance. The model is highly flexible, ranging from five-minute conversations to more in-depth sessions.

INPUTS	ACTIVITIES	OUTPUTS	SHORT TERM OUTCOMES	LONG TERM OUTCOMES
• Trained frontline staff • eLearning/workshop programmes • Validated tools (AUDIT, ASSIST)	• Support • Ask & Assess • Offer Assistance • Refer	• Number screened • No.of BI delivered • No.of onward referrals to Tier 3/4	• SMART goals agreed • Readiness to change increased • Initial consumption reduction	• Reduced Mortality • Reduced ED re-attendance • Sustained reduced consumption

Evaluation Framework

A logic model is a visual framework that systematically links program resources and activities to their outputs and intended short- and long-term outcomes, and it was used here to establish a structured foundation for evaluating both the implementation process and overall effectiveness of the SAOR model.

Process evaluation questions

- Are practitioners across Tiers 1–2 actually using "teachable moments" as intended, or are these opportunities missed?
- Is the SAOR conversation being delivered flexibly (5 min vs 1 hr) in a way that fits real clinical workflow, or is time pressure limiting Ask/Assess/Offer/Refer to a checklist?
- Are referral pathways from Tier 2 into Tier 3/4 actually cohesive, or do patients fall through gaps?

Outcome evaluation questions

- Do AUDIT/ASSIST scores show a measurable reduction post-intervention?

- Is there a measurable drop in ED re-attendance among those who received SAOR vs those who didn't?
- Are outcomes equitable across subgroups (e.g. do rural patients or non-native English speakers benefit equally)?

Methods

This evaluation employed a mixed-methods approach—combining quantitative analysis of screening and outcome data with qualitative thematic analysis of practitioner and patient perspectives—to comprehensively address the core process and outcome questions set out in the framework. Integrating these complementary methods allowed for a robust assessment of both measurable intervention effects and the contextual factors influencing real-world implementation. It should be noted that all datasets and interview quotes presented throughout this evaluation are illustrative and synthetic, created specifically to demonstrate methodology and analytical structure rather than represent empirical health system findings.

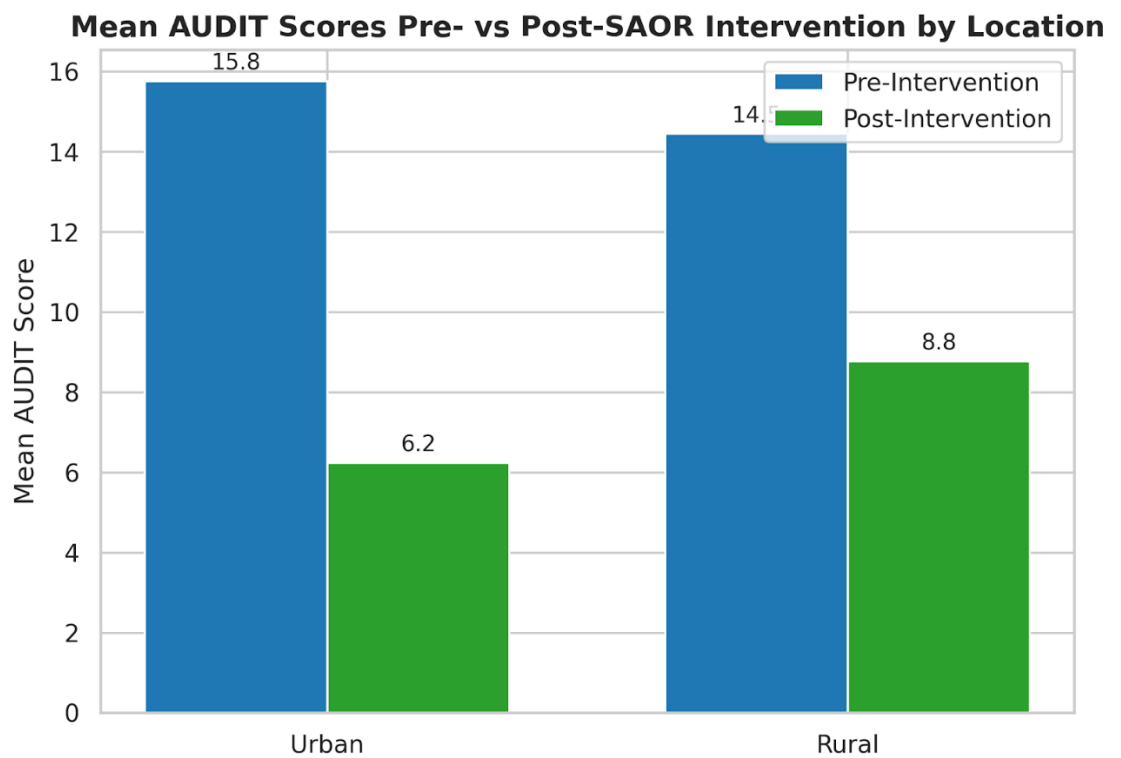
Quantitative Findings

Metric	Overall	Urban	Rural
Mean AUDIT Score (Pre)	15.2	15.8	14.5
Mean AUDIT Score (Post)	7.3	6.2	8.8
Mean AUDIT Reduction	-7.9	-9.5	-5.7
6-Month ED Re-attendance Rate	23.3% (76.7% no re-attendance)	—	—

Across all evaluated patients, mean AUDIT scores decreased substantially post-intervention from 15.2 to 7.3. However, urban patients experienced a greater average score reduction (-9.5 points) compared to rural patients (-5.7 points).

Overall ED re-attendance within 6 months post-intervention remained low at 23.3%.

Below is the comparative chart illustrating pre- and post-intervention AUDIT scores broken down by location:



The illustrative data suggest meaningful average improvement in AUDIT scores post-intervention, though this improvement was smaller among rural patients—a pattern warranting further investigation into potential structural barriers to follow-up care in non-urban settings.

Qualitative Findings

Quote (Paraphrased / Illustrative)	Theme
1. Patient worrying about being judged after the questionnaire was pulled out	Stigma & Discomfort

Quote (Paraphrased / Illustrative)	Theme
2. Patient struggling to get to follow-ups due to limited rural buses	Access & Transport Barriers
3. Patient appreciating a conversation without lectures or scary stats	Non-Judgmental Communication
4. Patient reflecting on the practitioner's chart during later drinking decisions	Delayed Behavioral Impact
5. Patient shutting down when caught off guard during a routine appointment	Unexpected Intervention Friction
6. Staff feeling rushed with waiting patients and tight appointment slots	Time & Resource Constraints
7. Staff navigating grey areas on when brief talk vs. Tier 3/4 referral is needed	Referral Pathway Ambiguity
8. Staff blanking on textbook phrases during intense patient reactions	Training vs. Real-World Capability Gap
9. Staff feeling validated when returning patients report long-term success	Professional Reward & Motivation
10. Staff deprioritizing screening in favour of immediate physical health issues	Competing Clinical Priorities

These qualitative themes directly address the primary process evaluation questions by surfacing the practical barriers practitioners and patients encounter during real-world SAOR delivery. The themes of *Time & Resource*

Constraints and Competing Clinical Priorities demonstrate why practitioners struggle to capture "teachable moments," as overwhelming workloads often reduce the "Ask, Assess, Offer Assistance" stages down to a rushed, checkbox exercise rather than a flexible 5-to-60-minute conversation

Equity Considerations

The evaluation findings highlight those structural barriers, such as inadequate rural public transit, prevent non-urban populations from benefiting equally from post-intervention care and follow-up support. This logistical friction, paired with the observed rural/urban gaps in AUDIT score outcomes, demonstrates that individuals in isolated or rural communities are currently at risk of being under-reached by standard SAOR pathways. To address these disparities and make delivery truly equitable, service planners should directly involve rural service users and community transport initiatives through participatory focus groups to co-design flexible, mobile, or telehealth-supported brief intervention models.

Recommendations

- **Clarify Referral Pathways:** Develop clearer triage guidelines and decision-support algorithms to eliminate ambiguity for staff when deciding whether to step patients up from Tier 2 brief interventions to Tier 3/4 specialist services.
- **Expand Access Supports for Rural Patients:** Address geographical equity gaps by piloting flexible delivery options—such as telehealth follow-ups or integration with community transport—to improve attendance and post-intervention outcomes for non-urban populations.
- **Enhance Practical Training Models:** Supplement existing eLearning modules with interactive, scenario-based workshops focused on managing time pressure, handling patient defensiveness, and navigating real-world emotional distress.

- Streamline Screening Workflows: Integrate automated screening prompts and rapid assessment templates within electronic health records to reduce administrative burden and help practitioners capture "teachable moments" during busy clinical shifts.

Reflective note

I put together this evaluation framework as a self-directed demonstration of my analytical approach and active interest in service evaluation, specifically designed to showcase how I approach implementation science and outcomes measurement ahead of applying for the SAOR EBI evaluation post. By combining a structured logic model, quantitative metric analysis, and thematic qualitative synthesis, this portfolio piece reflects my capability to bridge frontline healthcare insights with rigorous health service methodology.